

ZONTRON PHARMACEUTICALS SDN. BHD. (445695-T)

Lot 10 & 11, PERDA Industrial Park, **Lorong IKS Simpang Ampat B**, 14100 Simpang Ampat, S.P.S., **Pulau Pinang**, Malaysia.

PRODUCT LITERATURE**ERYBION CAPSULE****Each capsule contains**

Ferrous Fumarate BP	300 mg	Cyanocobalamine BP	50 mcg
Ascorbic Acid BP	100 mg	Pyridoxine HCl BP	10 mg
Folic Acid BP	2.5 mg		

Description

Hard gelatin capsule, telescope type, #1, maroon/maroon coloured.

Pharmacology

Iron : It's an essential component in the physiological formation of hemoglobin. Ferrous fumarate is regarded as an easily tolerated, well absorbed and adequately utilised form of oral iron medication.

Pyridoxine HCl : Although anemia is not usually a feature of true or conditioned pyridoxine deficiency, occasionally, megaloblastic erythropoiesis occurs. Hypochromic anaemia refractory to treatment with iron given orally or parenterally may respond well to the oral administration of pyridoxine HCl.

Vitamin C : Necessary for haemopoiesis. Aids the absorption and utilization of iron and plays a significant role in the maturation of red blood cells.

Vitamin B12 and Folic Acid : Both are necessary to prevent or reverse megaloblastic arrest of bone marrow and the resulting macrocytic hyperchromic peripheral blood. Neither vitamin B12 **nor** folic acid appears to be effective in the complete absence of the other.

Indication

For the management of anaemias.

Contra-Indication

This medication is not indicated in the treatment of pernicious anaemia.

Drug Interaction

This medication may interact with the following when taken concurrently :

Acetohydroxamic acid – iron are chelated resulting in reduced intestinal absorption of acetohydroxamic acid and iron supplement.

Antacids, calcium supplements, coffee, eggs, foods/medication containing bicarbonates, carbonates, oxalates or phosphates, milk or milk products, tea containing tannic acid, whole grain bread/cereal or dietary fiber, Cimetidine, oral tetracycline, penicillamine or trientine, zinc supplements, etidronate, fluoroquinolones, deferoxamine and other chelating agents

- decrease iron absorption because of formation of less soluble or insoluble complexes. Iron supplement should be taken 2 hours after ingestion of the above.

Coumarin or indandione-derivative anticoagulants – doses of 10 gm or more a day of ascorbic acid have been reported to impair gastrointestinal absorption of the anticoagulant.

Cellulose sodium phosphate – resulted in the metabolism of ascorbic acid to oxalate.

Deferoxamine – enhance tissue iron toxicity.

Disulfiram – interfere with disulfiram-alcohol interaction.

Long-term use of analgesics, carbamazepine, estrogens and oral contraceptive – requirement for folic acid and pyridoxine may be increased.

Hydantoin anticonvulsants – decrease the effect of hydantoin anticonvulsant by antagonism of their central nervous system effects.

Aluminium or magnesium-containing antacids – decrease folic acid absorption by lowering pH of the small intestine.

Antibiotics – interference with the microbiologic method of assay for serum and erythrocyte folic acid and cyanocobalamine concentrations and cause falsely low results.

Cholestyramine – interfere with the absorption of folic acid

Methotrexate, pyrimethamine, triamterene or trimethoprim – act as folate antagonists by inhibiting dihydrofolate reductase.

Sulfonamides, sulfasalazine – inhibit absorption of folate.

Zinc supplements –folate may decrease the absorption of zinc.

Cycloserine, ethionamide, hydralazine, immunosuppressants such as azathioprine, chlorambucil, corticosteroids, corticotropin, cyclophosphamide, cyclosporine and mercaptopurine, isoniazid, penicillamine – pyridoxine antagonist or increasing renal excretion of pyridoxine.

Levodopa – concurrent use is not recommended since levodopa's antiparkinsonian effects are reversed by pyridoxine.

Excessive intake of alcohol, aminosaliclates, colchicines in combination with aminoglycosides – reduce absorption of cyanocobalamine from the GIT.

Warning / Precautions

Erybion capsule may alter the absorption of some drugs, for example antacids and tetracyclines. Hence if it is necessary to administer these drugs together, they should be taken 2-3 hours apart.

Adverse effect

Abdominal or stomach pain, cramping or soreness, contact irritation due to contact with ulcerous areas or high concentration of iron in one area, constipation, diarrhea, nausea, vomiting, darkened urine, heartburn and staining of teeth.

Incidence dose-related : Kidney stone / oxalate (side or lower back pain), allergic reactions (bronchospasm, erythema, fever, skin rash or itching), sensory neuropathy.

Dosage

Adult : 1-2 capsules a day or as directed by physician

Children (6-12 years) : 1 capsule daily

Symptoms and Treatment for Overdosage

Iron toxicity : Diarrhea, sometimes containing blood, fever, severe nausea / vomiting, sharp stomach pain / cramping, bluish-coloured lips / fingernails / palms of hand, drowsiness, pale clammy skin, seizures, unusual tiredness or weakness, weak and fast heartbeat.

Treatment : Induce emesis with syrup ipecac or lavaging with sodium bicarbonate (if patient is comatose or having convulsion). Fluid and electrolyte balance must be maintained. Deferoxamine may be administered slowly, intravenously or intramuscularly in more severe iron toxicity.

Packing

Packed in plastic container of 500's capsules.

Pharmaceutical Precautions

Store below 30°C in a dry place, protected from **direct** light.

Keep away from reach of children.

Expiry date: 2 years from date of manufacture

Name and Address of Manufacturer

TERAPUTICS SDN. BHD. (590500-W)

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14100 Simpang Ampat, S.P.S.,

Pulau Pinang, Malaysia.

Name of Distributor

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Malaysian **Drug** Registration No. :MAL19950121XC